

Serum free light chain assay & Serum protein electrophoresis due in November-2025.

For Scheduling Appointments please contact: Call Centre (0172-5212000 Or Please visit the front-office to get your next appointment booked.

Do not call Doctors mobile for appointment)- Call Appointment desk. Between 9 am to 5 Pm Call in Oncology Day Care for any Emergency/Or any otherquery (2856)

In Emergency please contact: 2666 / 77107-77107(Max Hospital Emergency No) Dr. Sachin Gupta (6280334490), Dr Krutika 7988689148

To Send Email to Shama@maxhealthcare.com and Whatsapp: 7973934490 (9:00 AM to 6:00 PM)

I _____ have understood and have been given an opportunity to clarify my concerns in the language I/we understand & hereby acknowledge receipt of the discharge summary.

Vipin Kumar Garg

Patient/Attendant

Nurse

IN CASE OF EMERGENCY:-

Patient Name :

SSN No. : 150435714

/es/ Ladi Saluja
Medical Transcriptionist - L1

Signed: 21 AUG, 2025 12:57

/es/ Ladi Saluja
Medical Transcriptionist - L1

Cosigned: 21 AUG, 2025 12:57
for Sachin Gupta
Director

Entered Date : 21 AUG, 2025 11:33

Prepared By: Ladi Saluja

YEARS OF
SERVICE AND
EXCELLENCE

For post-hospital care at home, call Max@Home at 8744 888 888 (24x7 helpline). Services include Critical Care@home, Nursing Healthcare Attendant, Physio-therapy, X-Ray, Sample Collection, Medicine Delivery, Medical Equipment and more.

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Hexidine mouthwash 10 ml oral rinse three times daily to continue, preferably after meals.

Plenty of water

SOS LIST

Tab Normaxin 1 tablet SOS for abdominal cramps

Movicol Sachet 1 sachet twice daily / sos for constipation

Tab Flexon 1 tablet SOS for body aches

Tab Dulcolax 2 tab sos for constipation

Tab Dolo 650 mg 1 tablet SOS for pain/fever

Syp Cremaffin 10 ml SOS at bed time for constipation

SSN No. : 150435714 Patient Name : Vipran Kumar Garg

Special Needs:

Special Precautions

If you experience any of the following, report to emergency room at Max Super

1. Severe weakness /Severe mouth ulcers
2. Fever
3. Loose stools/motions/vomitings
4. Bleeding from any site or passing black stools like coal tar.
5. Chest pain, breathing difficulty, pain in abdomen.
6. Reduced urine output.
7. Rash over skin, swelling over body
8. Any other problem for which you think urgent attention is required.

Follow Up Advice:

Please follow up in Oncodaycare with Dr. Sachin Gupta/Dr. Krutika Goel on 18.09.2025 with CBC, KFT, LFT & RBS for chemotherapy.

Inj Denosumab 120mg s/c stat due on November-2025.

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Discharge Medication Advice:

Tab Pomalidomide 2 mg one tablet once daily 3 weeks on and 1 week off

Tab Pantop 40 mg 1 tablet once daily (Before Breakfast) x 3 days then SOS for acidity

Tab Emeset 8 mg 1 tablet three times daily x 3 days then SOS for Nausea/Vomiting

Tab Ecosprin 75 one tablet at bed time to continue.

Tab Shelcal one tablet once daily to continue.

Tab Acivir 400mg one tablet twice daily to continue.

Tab Thyronorm 50 mcg 1 tablet once daily to continue

Tab Torsemed 20 mg one tablet once daily before breakfast to continue.

Tab Nicardia XL 30 mg one tablet twice daily before breakfast and before dinner to continue.

Tab Arkamin 0.1 mg one tablet thrice daily after meals to continue.

Tab Metoprolol SR 50 mg one tablet after breakfast to continue.

Tab Silodocase D8 one tablet at bed time to continue.

Tab Irobor LP one tablet before lunch to continue.

Tab Atarax 25 mg one tablet once daily to continue.

Tab Auxisoda EC 1000mg one tablet twice daily before breakfast and before dinner to continue.

Tab Dytor 10 mg one tablet SOS

Syp Cremaffin Plus 15ML at bed time for constipation x 3 days.

K-Bind sachet, one sachet thrice daily in full glass of water x 3 days.

Foracort puff 2 puff at bed time

Patient Name : Vipin Kumar Garg
SSN No. : 150435714

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Patient was on Maintenance Daratumumab + Tab Pomalidomide from Jan 2025.

Plan:-

Inj Daratumumab 1200 mg D1Q monthly
Inj Denosumab 120 mg s/c stat3 monthly
Cap Pomalidomide 2 mg 1 cap 3 weeks on & 1 week off

Presenting Complaints:

Admitted for chemotherapy

Past Medical History:

Comorbidities

Hypertension
Hypothyroidism
H/O Bronchial Asthma since 20 years

Course in Hospital:

Chemotherapy Cycle 20 Day 1 on 21.08.2025:-(Maintenance Daratumumab)

Inj Daratumumab 1200 mg s/c
Inj Denosumab 120 mg s/c
with premedication

SERUM PROTEIN IFE ELECTROPHORESIS & Serum Free light chain assay done in august outside - report awaited

Patient tolerated chemotherapy well and is being discharged with below mentioned medication advise---

On Examination:

Pallor - Icterus -
CNS : Conscious, Oriented
CVS : S1S2 heard
Resp: Bilateral Ai entry equal
P/A : Soft, Non-tender, No Hepatosplenomegaly

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No Lymphadenopathy

Past Investigation :

MOLECULAR/ PREDICTIVE/ TUMOR MARKERS

02.05.2024

Free Kappa 20.82

Free Lambda 11.80

Kappa/Lambda, Ratio 1.7600

SERUM PROTEIN IFE ELECTROPHORESIS 02.05.2024

"M" Spike seen in gamma globulin region is of IgG kappa type.

SERUM PROTEIN ELECTROPHORESIS 03.05.2024

Hypoproteinemia, Hypoalbuminemia and hypogammaglobulinemia present.
Monoclonal gammopathy ("M" Spike 0.17 g/dl) seen in gamma globulin region.
(In a follow - up case).

22.07.2024

Free Kappa 18.66

Free Lambda 17.14

Kappa/Lambda, Ratio 1.089

SERUM PROTEIN IFE ELECTROPHORESIS 22.07.2024

Hypoproteinemia present
No "M" spike seen.
Hypogammaglobulinemia present.

29.11.2024

Serum protein profile by high-resolution Agarose gel electrophoresis reveals the presence of a faint non-discrete protein band in gamma globulin region, which could be 'M' spike.
Immunofixation electrophoresis (IFE) identifies the very faint non-discrete protein band (?) in gamma globulin region of IgG and Kappa lane corresponding to suspicious 'M' spike.

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IgG -6.84
IgM-< 21.0
IgA-< 33.0

29.11.2024

Free Kappa 20.31
Free Lambda 7.25
Kappa/Lambda, Ratio 2.801

Protein Electrophoresis 24 hour urine
Total Protein - 0.14
Total Urine Volume - 2400

10.02.2025

Free Kappa 46.57
Free Lambda 15.04
Kappa/Lambda, Ratio 3.096

SERUM PROTEIN IFE ELECTROPHORESIS 10.02.2025

M Spike - ? 0.2

Suspicious non-discrete protein band in gamma globulin region, possibly monoclonal gammopathy.

24.05.2025

Free Kappa 48.70
Free Lambda 19.25
Kappa/Lambda, Ratio 2.530

SERUM PROTEIN IFE ELECTROPHORESIS 24.05.2025

M Spike - ? 0.2

Suspicious non-discrete protein band in gamma globulin region, possibly monoclonal gammopathy.

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Page 7 of 8

Plan :-

Inj Carfilzomib 54mg/m² IV D1, D8, D15
Inj Daratumumab 1200 mg IV D1
Tab Pomalidomide 2 mg PO x 1 month
Inj Zoledronic 4mg IV monthly

In Sept 2024:- Patient presented with the complaint of anuric status and raised creatinine level (5.54) in view of aki. Patient was already undergoing treatment from Medanta hospital, Gurgaon for the same. Patient was admitted in medanta hospital with complaint of fever and cough and later had reduced urine output and was later anuric. Onco medicine consult was done and hemolysis workup was sent which revealed indirect coombs test positive. Dialysis was done in view of fluid overload regularly.

He was also found to have severe thrombocytopenia and anemia over there. In view of the above, differential diagnosis of HUS was also kept. Kidney biopsy done as he wasnt improving and remained anuric. Renal biopsy report awaited . Gradually improvement in urine output - 400ml /day .

Streptococcus Pneumonia

Already resolved by the time he was shifted to Max, Mohali.

USG ABDOMEN WITH PELVIS OUTSIDED ON 08.09.2024

-BOTH KIDNEYS SHOW MILD DIFFUSE INCREASED RENAL PARENCHYMAL ECHOTEXTURE

ECHO OUTSIDED ON 09.09.2024

NORMAL LV SYSTOLIC FUNCTION (LVEF=65%)
TYPE 1 DIASTOLIC DYSFUNCTION

SPUTUM ANALYSIS OUTSIDED ON 08.09.2024

STREPTOCOCCUS PNEUMONIA-DETECTED (CT VALUE-30)

HRCT CHEST DONE OUTSIDED ON 07.09.2024

MINIMAL BILATERAL PLEURAL EFFUSION WITH FISSURAL EXTENSION,PASSIVE ATELECTASISAND MILD SMOOTH INTERLOBULAR SEPTAL THICKENING OF THE UNDERLYING LUNG PARENCHYMA,FEW TINY FOCAL AREAS OF AIRSPACE CONSOLIDATIONS ARE SEEN IN BASAL SEGMENTS OF BILATERAL LOWER LOBES(RIGHT>LEFT)

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Erythropoiesis: Normoblastic erythroid reaction
Myelopoiesis: Normal sequential maturation
Megakaryopoiesis: Megakaryocytes are adequate in number and normal in morphology.
Lymphocytes: Normal in number and morphology
Plasma cells: Mildly increased in number constituting 3% of total cellularity, including reactive forms.

BONE MARROW BIOPSY

Microscopic: Bone marrow biopsy is adequate for evaluation showing bony trabecular with 10 intertrabecular spaces,
Cellularity: Normocellular (60-65%)
Report-Bone marrow biopsy shows trilineage hematopoiesis with normal distribution of hematopoietic cells.
Plasma Cells: There is no appreciable increase in plasma cells OR focal clustering
Impression: Bone marrow appears to be in morphological remission in a follow-up case of multiple myeloma.

PET CT SCAN ON 13/02/24

IMPRESSION: Case of multiple myeloma, post chemotherapy with increase in kappa FLC during follow-up. PET/CT reveals:

1. Metabolically active lytic lesions involving few skeletal sites as described (pathological fractures involving right 6 & left 11 ribs, collapsed D1 vertebra) - residual/recurrent neoplastic disease involvement.
2. Non-metabolic lytic lesions in multiple other skeletal sites as described above; D1 & D2 vertebral collapse with no significant metabolic activity.
3. Also noted:

(i) Metabolically active multiple irregular fibronodules, few arranged in tree-in-bud pattern with adjacent ground glass haziness in the left lung with multiple mediastinal & right supraclavicular lymph nodes - favors active infective/inflammatory etiology.

(ii) Hepatomegaly.

(iii) Prostatomegaly.

4. No definite scan evidence of abnormal metabolically active disease in rest of the body in this study.

Previous FDG PET/CT scan CD not available for comparison (done at PGI, Chandigarh, dated 10.08.2023).

HRCT CHEST ON 22/03/24

Impression: K/c/o multiple myeloma, Present scan shows:
Multiple centrilobular nodules and areas of consolidation in left lower lobe-infective - Multiple lytic lesions throughout the axial skeleton and right 6th rib with vertebra plana D6 with paravertebral soft tissue.
-Dilated thoracic esophagus with hiatal hernia

Then he was planned for chemotherapy with below mentioned plan in view of residual/recurrent neoplastic disease involvement.

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Page 3 of 8

Investigations

Serum free light chain assay (30/05/2022)

Kappa - 857.95

Lambda - 17.82

Kappa/Lambda Ratio - 48.15

Urine Protein (01/06/2022) - 922 mg/TV

PBF (01/06/2022) : Mild anisocytosis. Normocytic normochromic red cells admixed with a few macrocytes. Platelets are adequate with a few large forms. No circulating plasma cells or Rouleaux formation seen.

MRI spine Dorsal (contrast)(25/07/2022):

- Multiple enhancing dorsal vertebral body and posterior element osseous lesions with pathological collapse D1 and D6 vertebral bodies -c/o MM.
- Vertebra plana D6 with enhancing bilateral para vertebral soft tissue component showing extradural intra spinal extension with bilateral foraminal compression and involvement of bilateral costochondral rib junctions.
- Minimal bilateral pleural effusion.

CT Thorax / Chest - HRCT Plain of 19-SEP-2022:

- Few tiny ground glass nodules are seen scattered in the left lower lobe. ? infective (Adv follow up)
- Small lytic lesions in the multiple thoracic vertebral bodies with complete collapse of the D1 and D6 vertebrae.

Serum protein electrophoresis on 18/10/22 -Monoclonal gammopathy ("M" spike-0.06 g/dl) seen in gamma globulin region.

Serum protein electrophoresis on 16/02/23 -Monoclonal gammopathy ("M" spike-0.04 g/dl) seen in gamma globulin region

Serum protein electrophoresis on 23/05/2023 -no "M"spike seen

Serum protein electrophoresis on 30/12/23-Monoclonal gammopathy ("M" spike 0.24g/dl) seen in gamma globulin region

Received 4 cycles with Inj Bortezomib /Inj Daratumumab/ Inj Cyclophosphamide till 03.10.2022, Took Tab Endoxan 50 mg IN 2022.

BONE MARROW ASPIRATION AND BIOPSY ON 10/02/24

Peripheral Blood (Smear)

Findings: Peripheral smear shows normocytic, normochromic anemia with mild thrombocytopenia

BONE MARROW ASPIRATION and IMPRINT SMEAR REPORT (BM-35/24)

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Page 2 of 8

Discharge Summary Medical Oncology

Patient Demography Details :

Name : Vipan Kumar Garg	Patient ID : MHLI.435714	IP No. : 249646
DOB : 30 MAR, 1957	Age/Gender: 68 Years/MALE	Primary Consultant 1 : Sachin Gupta
DOA : 21 AUG, 2025 11:31	Ward : MHLI-LB-ONCODYCARE	Primary Consultant 2 : Krutika Kurhade
Address: HNO. 323 FF, SEC P PANCHKULA HR, Panchkula Industrial Area Phase 1, HARYANA, 134113	Mobile No. : 9815311118	

Date and Time of Discharge: 21 AUG, 2025 15:00

Diagnosis:

Multiple Myeloma
C- R- A- B+
ISS-II
RISS-II
Received 4 cycles with Inj Bortezomib /Inj Daratumumab/ Inj Cyclophosphamide till 03.10.2022
Relapse in Feb 24
Post 6 cycles with Inj Carfilzomib + Inj Daratumumab + Inj Zoledronic and Tab Pomalipomide till 04.09.2024.
Acute kidney injury in sept 2024.
Patient was on Maintenance Daratumumab + Tab Pomalidomide from Jan 2025.

Treatment Plan:

Plan:-
Inj Daratumumab 1200 mg D1Q monthly
Inj Denosumab 120 mg s/c stat3 monthly
Cap Pomalidomide 2 mg 1 cap 3 weeks on & 1 week off

Brief Summary :

Mr. Vipan Kumar Garg, 65 years gentleman was apparently alright till May end 2022 when he actually started having complaints of pain in back region and right side of chest for which he went to PGI, Chandigarh where he was evaluated and investigated.

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Page 1 of 8